

Trauma

T-05 Abdominal and Extremity Trauma BLS

PRIORITIES:

- ABCs
- Any penetrating injury – handle as if abdominal cavity penetrated;
- Check for exit wounds;
- Assure an advanced life support response..

*As with all traumatically injured patients, transport per **LOAD AND GO Procedure** with special considerations.*

General Treatment General Guidelines

1. Ensure a patent airway (suction as necessary)
2. Be prepared to support ventilation with appropriate airway adjuncts;
3. OXYGEN THERAPY – Begin oxygen 10 liters/minute by mask. DO NOT withhold oxygen from a patient in respiratory distress because of a history of COPD.
4. Nothing by mouth;
5. Do NOT allow the patient to move;
6. Anticipate vomiting and shock.

Impaled Object

1. Attempt to stabilize the object with bulky dressings. Do NOT remove unless object interferes with CPR (Consult ALS unit staff as soon as possible).

Eviscerating Trauma

1. Cover eviscerated organs with sterile saline soaked dressing and firm bandage;
2. Do NOT replace organs into abdominal cavity.

Genital Injury

1. Cover genitals with sterile saline soaked gauze;
2. Treat amputated parts per EXTREMITY AMPUTATIONS (Protocol t-6)
3. Apply direct pressure to brisk bleeding.

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Trauma

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PRIORITIES:

- ABCs
- Determine baseline sensory and motor function/deficits prior to skeletal stabilization/splinting;
- Determine if patient's injury(s), history, mechanism of injury requires "LOAD AND GO" activities;
- Assure an advanced life support response..

*As with all traumatically injured patients, transport per **LOAD AND GO Procedure** with special considerations.*

Extremity Trauma General Guidelines

1. Return extremity to anatomic position if possible as resistance/pain allows;
2. Apply splints and re-check neurovascular status after each manipulation and periodically enroute.
3. Control bleeding with direct pressure;
4. Cover open fractures with sterile saline soaked gauze;
5. Splint all dislocations (joint injuries) in position found and transport as soon as possible.

Amputations

1. Care of the amputated extremity:
 - If partial amputations, splint in anatomic position and elevate the extremity;
 - If the part is completely amputated, place the amputated part in a sterile dry container or bag. Seal or tie off the bag if possible. Place in a second container or bag if possible and seal or tie off. Place on ice if available. DO NOT place part directly on ice or in water. Elevate the extremity involved and dress in dry gauze.

Hand and Wrist

1. Splint to include wrist;
2. Assess distal function (pulse, color, sensation and motion) before and after splinting.

Elbow Dislocation

1. Splint in position found;
2. Assess distal function (pulse, color, sensation and motion) before and after splinting

Upper Arm

1. Splint and swathe arm;
2. Assess distal function (pulse, color, sensation and motion) before and after splinting

Shoulder Fracture and Dislocations

1. Splint in position of comfort, sling and swathe as warranted;
2. Assess distal function (pulse, color, sensation and motion) before and after splinting

Clavicle

1. Sling and swathe arm

Trauma
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Scapula

1. Sling for arm
2. Assess respiratory status;

Ribs

1. Assess respiratory status
2. Flail chest assessment (See Chest Trauma, Protocol t-4)

Pelvis

1. Place on long board immobilization device;
2. Do not roll patient;
3. Treat for shock, if present;
4. Splint legs together, padding under knees for comfort;

Femur

1. Splint using an appropriate traction device or fixation splint, if not accompanied with a pelvis and/or lower leg fracture;
2. Assess distal pulses and neurologic status before and after splinting;
3. Treat for shock, if present.

Fibula-Tibia

1. Splint adjacent joint
2. Assess distal pulses and neuro status before and after splinting
3. Treat for shock, if present

Hip Fracture or Dislocations

1. Stabilize in position of comfort
2. Assess distal pulses and neuro status before and after splinting
3. Treat for shock, if present.

Knee Fractures and Dislocations

1. Splint in position found;
2. Assess distal pulses and neuro status before and after splinting

Foot and Ankle Fractures

Splint but do not apply traction splint;
Assess distal pulses and neuro status before and after splinting

Jaw (Maxillo-Facial Trauma)

1. Maintain airway;
2. Suction as necessary
3. Consider c-spine immobilization
4. Position patient to maintain airway
5. Collect avulsed teeth and place in moist sterile gauze and plastic bag.

Trauma

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ABDOMINAL TRAUMA

IMPALED OBJECT

- Attempt to stabilize the object
- If object interferes with CPR, consult Base Hospital

EVISцерATING TRAUMA

- Cover eviscerated organs with sterile, saline-soaked gauze
- DO NOT replace organs in the abdominal cavity

GENITAL INJURY

- Cover genitals with sterile, saline-soaked gauze
- Treat amputated parts per extremity amputations
- Apply direct pressure as needed to control active bleeding

Extremity Trauma/Amputation

Return extremity to anatomic position, if possible and resistance/pain allows.

If extremity is dislocated, splint in position found.

Control bleeding with direct pressure.

Refer to S-02 Wound Packing and S-03 Tourniquet, if needed.

If bleeding cannot be controlled, or massive hemorrhage suspected, refer to T-06 Massive Hemorrhage.

If partial amputation, splint in anatomic position and elevate

COMPLETE AMPUTATION

- Place amputated part in a dry, sterile, sealed container or bag. Place container/bag in a second container on ice if available.
- DO NOT place part directly on ice or in water.
- Elevate the involved extremity and attempt to achieve homeostasis.

For Isolated Extremity Injury (including hip and shoulder)

Refer to M-04 Pain Management

DISRUPTED COMMUNICATIONS

In the event of a "disrupted communications" situation where a base hospital physician CANNOT be contacted for orders, Solano County Paramedics MAY NOT utilize the portions of this protocol requiring base physician orders AND must transport to the closest receiving facility.